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Evaluating Paternalistic Health Policies

In the introduction of his book *Burn Out Society*, German-Korean philosopher Byung-Chul Han lays out the paradigms of disease over the course of human history. For a majority of its existence, humankind has found itself plagued by what Han calls "negative pathologies" – health outcomes caused by a lack of some preventative faculty, such as malnutrition or unsanitary habitats. The medical community overcame these diseases, beginning at a macro level and moving down to the micro – from ensuring basic needs are met, to discovering antibiotics, to inventing vaccines and the executing the eradication of smallpox, and finally offering the possibility of genetic modification against hereditary diseases. Medical success in these areas, along with broader sociotechnical developments, have thrust contemporary humans in developed nations past these negative pathologies into a novel medical paradigm with positive pathologies of excess. Han attributes the illnesses that hallmark this era – depression, anxiety, lifestyle diseases – to overconsumption, overstimulation, and hyper-freedom – all in the excess. These "evil habits" account for 60 percent of (preventable) deaths in the United States (National Institutes of Health.) Public and private interests, motivated by an opportunity to do good and minimize the social burdens of poor health, implement policies to alleviate or eradicate these pathologies, although devising systems which preserve the virtue of individual liberty while balancing encouragement towards healthy decision-making remains a contested topic of ethical discussion. It is commendable for an employer to seek to reduce the

proportion of preventable health diseases among its employees, yet the employer must do so in a manner that limits dangerous perverse incentives, preserves personal liberties while accounting for diverse levels of agency, and is equitable throughout the social gradient. I seek to critique the following proposed policies of an institution based on these aforementioned values, as well as propose various paths an institution can take to achieve similar outcomes while more comprehensively aligning itself with these ethical pillars.

The proposed policies by the university faces drawbacks for instrumental reasons, namely in the erosion of trust between the institution and its employees via the intrusive measures required for the policies' executions. Any policy that requires smoking cessation must obtain proof of such cessation somehow – either through mandatory self-reporting or by obtaining health records from a physician. Even though dishonesty can be prosecuted as fraud, an individual may calculate that the risk of fraud is worth the benefit for better health insurance coverage. Furthermore, if the physician route is taken, obtaining medical records may be an intrusion of privacy unwelcomed by most employees. Additionally, it incentivizes individuals to be dishonest about their own health with their doctor in order to keep that information from even reaching their medical records in the first place, therefore endangering the individual by providing their doctor with incomplete information. This health danger is exactly what the university claims to seek to prevent with these policies – to act otherwise contradicts this motivating tenet.

An additional instrumental shortcoming lies within the inequities of these policies with respect to individuals of lower socioeconomic privilege. As extensive literature has confirmed, there exists a social gradient of health outcomes, and an established theory for these disparities is the greater prevalence of lifestyle diseases among lower socioeconomic classes. The proposed

policies target these diseases by offering rewards in the form of premium healthcare coverage – a metaphorical "carrot" – in exchange for employees' remedying these diseases through means such as BMI targets or smoking cessation. Oftentimes, however, the failure of individuals to successfully achieve these measures penalizes them by this withholding of health insurance. Individuals of lower socioeconomic status are less able to afford their own health insurance plans, and thus rely on employment for this coverage. This creates a "double jeopardy" among these groups; Not only are they more likely to suffer from lifestyle diseases, but they are also less likely to be able to afford health insurance to cover the resulting negative outcomes of lifestyle disease and are furthermore penalized for a disease of which they have a nebulous amount of responsibility for contracting. The "carrot" becomes akin to a "stick" in this framing.

The aforementioned instrumental criticisms stem from intrinsic values of an established ethical framework supported by a contemporary scientific paradigm. Regardless of the policies themselves, the underlying morality motivating their implementation must be for the sake of the good to justify any downstream strategies. These values must balance encouraging beneficial health outcomes with the preservation of individual autonomy and liberties. Practices such as "nudging" take advantage of subconscious psychological biases and heuristics to raise the probability of an agent making a targeted choice. These nudges can be as simple as defaulting options to promote long-term stability. Paternal institutions can justify these policies along a Socratic intuition that people do not make the wrong decisions due to evil intentions, but rather because of human foibles that cloud their reasoning, such as short-sightedness. Continuing this line of reasoning concludes that a lack of wisdom is the root of human error, and thus the institution, which positions itself in a narrative of intellectual superiority over any single individual, will make the best decisions for its employees in a paternal embodiment of Plato's

philosopher kings. Institutions often hedge this authoritarianism in exchange for the virtue of liberty by allowing employees to opt-out of default programs. This justification by the institution presupposes that it knows what is good for an individual better than the individual themself, who undeniably have a greater knowledge of their unique circumstances. Regardless of the possibilities of fallacious reasoning, this wealth of personal information could outweigh shortcomings in rationality. If an institution must establish some species of policy which is susceptible to choice architecture among employees, then the institution has a moral responsibility to sway such decision-making in favor of what it deems to be best for its employees. The alternative to this is undoubtedly immoral. However, these situations arise extremely rarely in the black-or-white form required to uphold this ethical standard, and more often than not there are a variety of alternative solutions that can be tailored to any situation.

The discussion of balancing the intrinsic values of liberty and social benefit in health outcomes arises from a metaethical foundation of deontology versus consequentialism. The consequentialist position is straightforward – paternalistic health policies are for the greater good of the individual, whether they know it or not. The deontological basis, however, must grapple with the validity of the underlying assumption of the role personal responsibility plays in morality. A champion of this position may argue that the inequitable outcomes of the aforementioned instrumental critiques of paternalistic policies are a result of poor decision-making by an autonomous agent, and thus not only is it ‘deserved’ that they face negative consequences, but it furthermore restricts their freedoms to prevent them from doing so. An a fortiori form of this argument may claim that in order for free will to hold any value in society, people must face consequences for their actions – a line of reasoning paralleling Christian theological arguments for the existence of evil. However, the narratives pushed by this

ideology have historically favored those in positions of social, capital, and institutional power. Between the explanations that this historical trend is either because privilege is more good than underprivilege or the ethical principle itself has been abused, the latter is a more agreeable position. If this is the case, then perhaps a principle so vulnerable to abuse is not the perfect ethical justification to be utilized under the social and material conditions of reality, especially if its abuse is an emergent property from this rigid socially organized substrate.

Under this analysis, liberty should not be appreciated as an ultimate good, especially if it can be twisted into a narrative to support an inequitable distribution of material conditions. Rather, a more consequentialist initiative should be deployed in weighing actual outcomes of these programs. Top-down institutions make arbitrary decisions frequently – not in a manipulative manner over individuals, but rather because social organization requires the establishment of norms and standards. Social contract theory dictates the relinquishing of some liberty for the sake of freedom in greater areas. Byung-Chul Han emphasizes how an excess of liberty creates its own soul-crushing cages. His example of affluent societies with soaring rates of burn out epitomizes this. The mental pressure to achieve success under the individualistic personal responsibility paradigm pushes a narrative that failure is only due to one's own decisions and inability to capitalize on their talents. It punishes healthier behavior such as enjoying hobbies with no intent to monetize them and spending spare time building deep relationships. Similarly, there is a middle-ground to inhabit for health policies that both provide resources to better the public while also being careful to reinforce a narrative that all responsibility for failures in health fall on the individual.